

Conversation #18

INTERMEDIATE

CONVERSATION

CRISIS MANAGEMENT – SERVICE FAILURE AT A HEALTHCARE NETWORK

Prepare 8 pieces of paper numbered from 1 to 8 and have each student draw one to discover their character.

This activity focuses on **crisis communication, prioritization, leadership language, and fast decision-making.**

It's **7:45 a.m. on a Tuesday** at **VitaCare Saúde Integrada**, a private healthcare network in São Paulo.

Overnight, the company's **appointment and patient record system went offline.**

As a result, clinics opened without access to schedules, patient histories, or insurance validations.

Patients are waiting, doctors are frustrated, and complaints are spreading quickly on social media.

The leadership team is called for an **emergency crisis meeting.**

Every minute without a decision increases **legal, financial, and reputational risk.**

Crisis Data

Indicator	Normal	Current Situation
Clinics operating	12	9
Appointments affected	0	410
Estimated revenue at risk	—	R\$ 520,000
Patient complaints (since 7 a.m.)	0	27
System recovery estimate	—	4–6 hours

Student Coaching – How to Use the Data

- Use the numbers to **decide priorities**
- Choose what to protect first: patients, staff, or systems
- Justify actions with **risk and impact**, not emotion
- Remember: healthcare crises involve **people, not only money**

Characters

1. Eduardo – Operations Director

"This situation is unacceptable.

Clinics are open, patients are waiting, and we're blind.

I don't need perfect solutions — I need decisions now.

Every delay makes this worse.

If we don't take control fast, this crisis defines our reputation."

2. Paula – IT Infrastructure Manager

“The system failure didn’t happen out of nowhere. We warned about server instability weeks ago. Right now, recovery is my priority. But rushing a fix could corrupt patient data. I need time — and no shortcuts.”

3. Ricardo – Clinic Operations Manager

“My teams are facing patients without answers. Doctors can’t access medical histories. We can operate manually for emergencies, not for everyone. If priorities aren’t clear, the clinics will collapse. I need direction, fast.”

4. Fernanda – Patient Experience Director

“Patients are scared and angry. Silence is the worst response we can give. We need clear communication — even if the news is bad. Trust disappears quickly in healthcare. I need approval to speak now.”

5. Helena – Finance Director

“This crisis already has a price tag. R\$520,000 at risk — and that’s conservative. Emergency actions cost money, but lawsuits cost more. We must track every decision financially. I want visibility, not surprises.”

6. Marcos – Compliance & Legal Advisor

“In healthcare, mistakes don’t end with apologies. Patient data and procedures are regulated. Manual processes increase legal risk. If protocols are broken, consequences come later. I will block any unsafe decision.”

7. Thiago – Medical Director

“Doctors are furious — and I understand why. Patient safety comes before systems and money. Some appointments must happen, no matter what. Others can wait. I will defend clinical priorities above all.”

8. Luana – Crisis Coordinator

“If everyone speaks at once, we lose time. My role is to organize chaos. I track decisions, responsibilities, and timing. At the end, this crisis must have a clear plan. Confusion is not an option.”

Writing #18

Full name: _____

COMPOSITION

Theme: Managing a Crisis in Healthcare

Guide your writing:

- 1 Describe the crisis and its impact.
- 2 Explain how the team reacted and prioritized decisions.
- 3 Present the final actions taken.
- 4 End with one lesson about responsibility or leadership.

Length: 120–150 words

Tone: professional and empathetic

Title: _____

[illegible]